

Pelvic Floor Therapy and Sexual Dysfunction

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Introduction

- Sexual dysfunction affects 43% of women in the general population.
- Sexual dysfunction is multifaceted and influenced by many psychosocial as well as physical factors.
 - Genito-pelvic pain disorder, dyspareunia, chronic pelvic pain, urinary incontinence, pregnancy, and prolapse may cause or influence
 - In women with dyspareunia – pain following intercourse often lasts 3 days

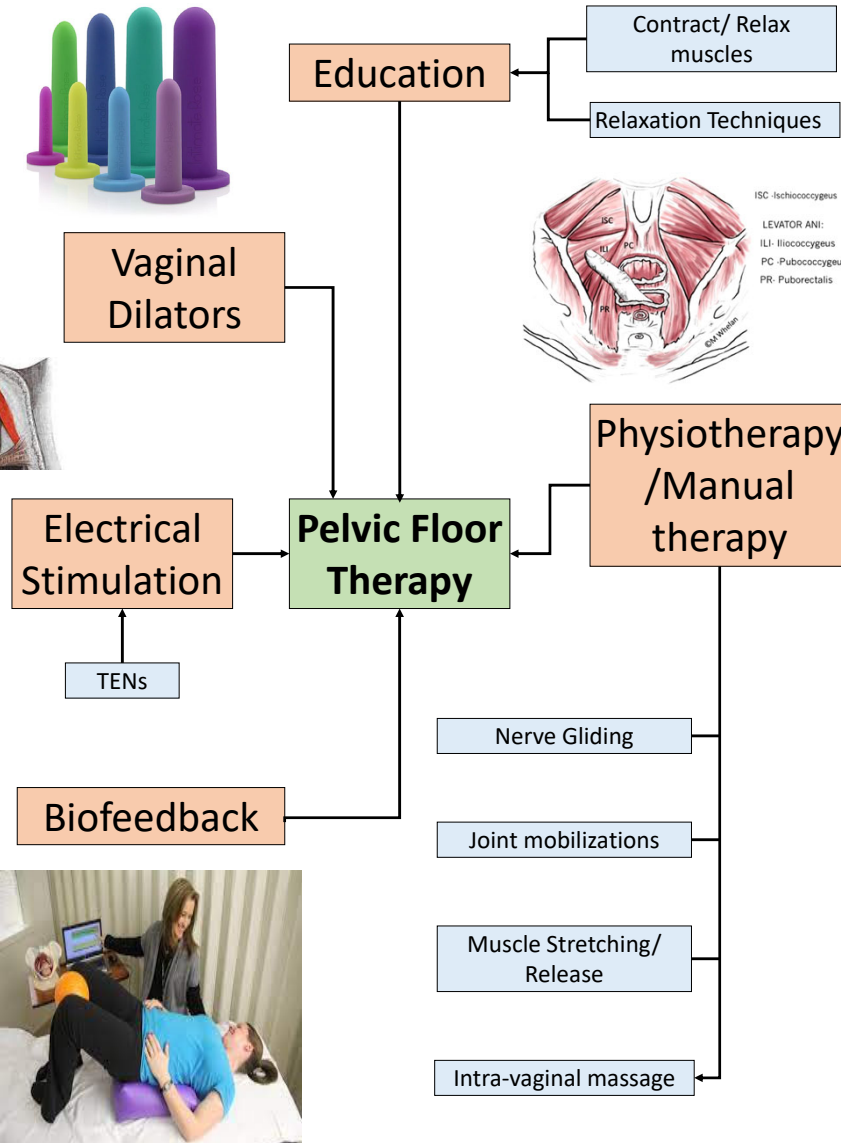
Pelvic Floor Therapy + Sexual Dysfunction

- Pelvic floor muscles (PFMs) are critical in:
 - Supporting organs
 - Bladder/bowel control
 - Achieving sexual arousal and orgasm
- Disturbance of PFMs can result in sexual dysfunction
 - ischioavernosus muscle (attaches to clitoral hood)
- Amount of sensation felt during intercourse based on strength of the PFMs
 - Rhythmic contraction = factor in arousal and orgasm
- Numerous studies indicate that pelvic floor therapy:
 - Decreases severity of sexual dysfunction
 - Improves dyspareunia, urinary incontinence, overactive PFMs, etc.
- Pelvic floor therapy uses a variety of different methods

Pelvic Floor Assessment

- History
- Assessment of pelvic floor muscles:
 - Digital palpation to assess tone (external, internal, intra-anal)
 - Flexibility of vaginal opening
 - Relaxation capacity
 - Tenderness/ Trigger Points
 - EMG to assess activity
 - Ultrasound
 - Dynamometry

Pelvic Floor Therapy Techniques



Mechanism of Action

- Education always first step
 - Anatomy and function – Relax and contract
- Physiotherapy/ Manual therapy
 - increase muscle tone/ circulation and release trigger points
 - Improve pelvic floor proprioception
 - Increase muscular strength and coordination
 - Critical smaller muscles – engorgement of clitoris
 - Nerve gliding – helps nerves that are restricted maintain normal function
- Vaginal dilators
 - relaxation of pelvic floor muscles
- Electrical stimulation
 - coordinate muscle contractions, decreased tenderness/tissue restriction
- Biofeedback
 - Education (digital and instrumental)
 - Decreased tenderness/tissue restriction

Key Points

- Education of healthcare workers
 - Increased need for pelvic floor therapy in certain patients
 - Stay up to date on latest peer reviewed pelvic floor research
- Pelvic Floor Therapy techniques are multi-disciplinary
 - Can include physical therapist, PCP, OBGYN, WHNP, CNM, urologist, sex or mental health therapist, etc.

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